

AFib Anticoagulation Restart Timing After Acute Ischemic Stroke

CLINICAL EFFICACY & SAFETY

RCT and individual-patient meta-analysis data support early DOAC initiation in carefully selected AFib-related ischemic stroke patients, especially mild-to-moderate infarcts without high-risk hemorrhagic transformation. Early treatment has not shown excess symptomatic intracranial hemorrhage (sICH) versus delayed treatment and may reduce recurrent ischemic stroke; DOACs are preferred over warfarin for most nonvalvular AF patients when anticoagulation is indicated.

1. Stroke Severity Classification (ELAN Imaging Criteria)

Minor / Small Infarct

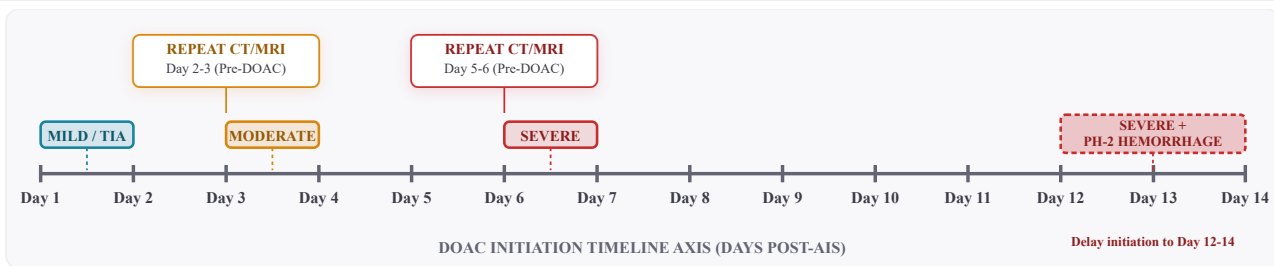
- TIA or infarct ≤ 1.5 cm on brain imaging. NIHSS can guide bedside risk but is not the ELAN definition.

Moderate Infarct

- Cortical superficial-branch lesion, internal border-zone lesion, or deep-branch lesion > 1.5 cm.

Major / Large Infarct

- Complete vascular territory, ≥ 2 moderate lesions, large multilobar infarct, or brainstem/cerebellar lesion ≥ 1.5 cm.



2. Bedside DOAC Dosing & Adjustment Guide

Drug	Standard Dose	Dose Reduction Criteria
Apixaban (Eliquis)	5 mg BID	Reduce to 2.5 mg BID if ≥ 2 criteria are met: • Age ≥ 80 years • Weight ≤ 60 kg • Serum creatinine ≥ 1.5 mg/dL
Rivaroxaban (Xarelto)	20 mg daily (with food)	Reduce to 15 mg daily if CrCl is 15–50 mL/min. Hold if CrCl < 15 mL/min.
Dabigatran (Pradaxa)	150 mg BID	Reduce to 75 mg BID if CrCl is 15–30 mL/min. Avoid if CrCl < 15 mL/min.
Edoxaban (Savaysa)	60 mg daily	Reduce to 30 mg daily if CrCl is 15–50 mL/min or weight ≤ 60 kg. Avoid if CrCl > 95 mL/min (high renal clearance reduces DOAC level).

ELAN Trial: Fischer U et al. *N Engl J Med.* 2023;388:2411-2421. | **CATALYST Meta-Analysis:** Dehbi HM et al. *Lancet* 2025. Early DOAC (median Day 2) vs delayed (median Day 7-8) showed no excess sICH (0.4% vs 0.4%) and fewer recurrent ischemic events in pooled data.

AFib Guidelines: Joglar JA et al. 2023 ACC/AHA/ACCP/HRS Guideline. *Circulation.* 2024;149:e1-e156.